

Prayers of Care Inc

Job Description — Peer Support Specialist

CARF Community Integration (BH) Binder — document JD-02

DRAFT — no approval date, no effective date. Do not date until signed.

Approver: Nicky Ingram, Chief Executive Officer / Owner
Community Integration, peer delivery model
No medication management · no faith-based activity
Status: DRAFT — not yet adopted, not yet effective
Prepared with Successful Solutions

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Status	Draft → Approved → Trained → In use → Tested → Monitoring *(circle the current state)*
Owner of this document	Nicky Ingram, Chief Executive Officer / Owner
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Next review due	_____
Legal entity	Prayers of Care Inc
CARF areas served	1.I Workforce Development and Management (p.80); 2.A Program/Service Structure (p.116); 2.C Person-Centered Planning (p.141); 3.C Community Integration (p.185)

1. Position

Title	Peer Support Specialist
Reports to	Program Supervisor — `[FILL IN name]`
Program	Community Integration (Behavioral Health), peer delivery model
FLSA / employment status	`[FILL IN]`
Work settings	The community, the homes of persons served, and the agency office at `[FILL IN address]`
Hours	`[FILL IN]`
Maximum active caseload	`[FILL IN]` — see the staffing plan for how the number was set

2. Purpose of the role

Walk alongside the people we serve. Use your own lived experience, ethically and on purpose, to support their recovery goals, their community participation, and the growth of their own natural supports.

You do not replace clinical treatment. You are a partner within the Community Integration program, and the person you serve leads.

3. Essential functions

These are the functions of the job for the purposes of the Americans with Disabilities Act. A qualified person must be able to perform them with or without reasonable accommodation.

Relationship and engagement

1. Engage people with respect, hope and cultural humility.
2. Build a mutual relationship in which the person leads and power is shared.

Planning and goals

3. Help each person develop their plan, with **goals recorded in the person's own words**.
4. Support wellness and recovery planning, including each person's crisis plan.
5. Review the plan with the person on schedule and record their own view of their progress.

Community participation

6. Connect people to community resources — housing, work, education, benefits, health care, recreation, transport, and community or faith groups **by the person's choice**.
7. Build and strengthen unpaid natural supports, and step back as those take hold.
8. Support advocacy, and teach self-advocacy so the person can do it themselves next time.

Lived experience

9. **Share lived experience purposefully and within a written disclosure plan.** See section 5.

Documentation

10. Document every service contact within `[FILL IN — hours]` of delivering it, personally.
11. Link every note to a goal on the person's current plan.
12. Write individualized notes. See section 6.

Safety and crisis

13. Recognize crisis, de-escalate, and make a warm hand-off to crisis services.
14. Report safety concerns and incidents promptly.
15. Follow emergency procedures as trained, on every shift you work.

Professional

16. Attend supervision and complete required training and competency assessment.
 17. Maintain the state peer credential and its continuing education.
 18. Follow all rights, confidentiality, boundary, safety and ethics policies.
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4. What this role does NOT do

Being explicit here protects you, the agency, and the people we serve.

- **No assessment, diagnosis, therapy or clinical treatment.**
- **No medication handling of any kind.** Prayers of Care Inc offers no medication management.

You do not give, hold, carry, sort, transport or dispose of anyone's medication, and you never advise a person to start, stop, change or skip a medication. What you **do** when someone raises medication is in section 7.

- **No legal or financial advice,** no signing documents for a person served, no acting as representative payee, no holding anyone's ID, benefits card, cash or bank access.
 - **No co-signing** a lease or a loan for a person served.
 - **No religious content introduced by you.** Prayers of Care Inc offers no faith-based activity. Spirituality is a life domain a person may choose; you follow their lead and never introduce prayer, belief or religious material unless the person has asked. See MVV-01, Statement on Faith.
 - **No service beyond your credential, training and assessed competency.** Refer instead, and tell your supervisor.
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5. Self-disclosure — the core tool of this job, and its most common failure point

Sharing your story is what makes peer support work. It is also the thing most likely to go wrong, so it is managed rather than left to instinct.

1. **You will write a personal disclosure plan with your supervisor,** recording what parts of your story you are prepared to share, what you will not share, and how you will answer something outside it. It is reviewed at least annually and any time your circumstances change.

2. **Before disclosing, ask yourself:** whose need does this meet? What do I want them to take from it? Is this the shortest version that does that? Am I about to give advice disguised as a story?

3. Disclosure is right when it conveys hope, normalizes an experience, shows a barrier can be overcome, or invites the person to say more.

4. Disclosure is wrong when it centres you, prescribes your pathway as the correct one, includes graphic detail, shifts support onto the person served, or fills a silence.

5. **Self-disclosure is discussed at every supervision session.** That is not surveillance — it is how the skill gets better.

6. In your first 90 days, your supervisor will directly observe you delivering service, including a disclosure.

7. In the record, a disclosure is documented as an intervention — **"shared lived experience of appealing a benefits denial to support the person's confidence in appealing"** — **never in personal detail**.

6. Documentation standards

This is where Medicaid audits are won or lost, so it is spelled out.

Every note must:

- be written by **you**, the person who delivered the service, within `[FILL IN]` hours;
- record the date, start and stop time, location and setting, and who was present;
- name **the goal it addresses, from the person's current signed plan**;
- describe **what you actually did**, concretely — not a label;
- record **how the person responded, in their own words where possible** — and this must not be a copy of what you wrote in the intervention;
- be signed with your name and credential.

Never: copy a previous note, copy another person's note, reuse the same wording across contacts, or record a service that did not happen as documented. If what you did does not fit a goal on the plan, that is a sign the plan needs updating — raise it, do not stretch the note to fit.

7. When someone raises medication

You are not qualified to advise, and you are also not required to ignore it.

1. Ask what is going on and listen.
2. Offer to help them contact their prescriber **the same day**, and help them write down what they want to say.
3. Go with them to the appointment if they want you there.
4. Document what the person told you and what you did.
5. Tell your supervisor.
6. If you see signs of a medical emergency — including a suspected overdose or a severe reaction — call emergency services immediately, give naloxone if you are trained and it is indicated, stay with the person, and file a critical incident report.

Never take medication from a person served for any reason, including to dispose of it.

8. Boundaries and dual relationships

Never, in any circumstance:

- a romantic or sexual relationship with a person served;
- lending, borrowing or giving money;
- a business or financial arrangement;
- accepting a gift beyond nominal value;
- providing housing to, or living with, a person served;
- using or being under the influence of alcohol or a non-prescribed substance during service.

Dual relationships are different, and they are expected. In a small recovery community you will meet people you serve at meetings, through friends, through your own history. That is not misconduct. **Not disclosing it is.**

- Disclose any dual relationship in writing before or immediately upon assignment, and no later than the next business day.
 - Your supervisor writes a management plan — reassignment, a written boundary agreement about what happens when you are both in the same room, or increased supervision.
 - **Your own recovery participation is never restricted by this job.** If you and a person you serve attend the same meeting, you agree in advance that neither raises the service relationship there. You are a member in that room, not staff.
 - Report the same day if a person served turns up at your home, contacts you personally, or offers a gift.
 - If you are unsure, raise it **before** you act. Asking is never held against you.
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9. Safety in the community — your right to leave

****You are never required to enter or remain anywhere you reasonably believe is unsafe.**

Withdrawing from an unsafe situation is never grounds for discipline. Not once, not ever.**

- Complete a visit safety plan with your supervisor before a first visit to a new location.
 - Record your itinerary before you leave, and keep it current.
 - Check in on arrival and check out on departure. If you are more than 30 minutes overdue and unreachable, your supervisor starts the escalation process — that is a protection, not a check on you.
 - Carry a charged phone. Ask for a two-person visit whenever you want one.
 - Move a meeting to a public place, the office or a phone call at your own discretion, and document why.
 - Do not intervene physically in any dispute. Leave, then report the same day.
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10. Your own wellness

This job is done by people in recovery, and it asks a lot. That is not a weakness in the design; it is the design. So:

- You will write a **Peer Wellness Plan** with your supervisor within 30 days of starting, saying what keeps you well, your own warning signs, and what you want your supervisor to do if those show. It is kept confidential and separate from your personnel file.
 - Time for your own recovery supports is accommodated in scheduling wherever operationally possible.
 - After a critical incident, a death, or anything that hits hard, support is offered within five business days. You may take it or decline it.
 - ****Asking for extra supervision, a caseload adjustment or time is not a performance concern and will not be recorded as one.****
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11. Core competencies

Trained **and** assessed. Attendance at a training does not establish competency; your supervisor assesses these by direct observation, review of your documentation, and skills demonstration, at 90 days and at least annually.

#	Competency
1	Engagement and building a mutual relationship
2	Purposeful self-disclosure within the written plan
3	Eliciting the person's own goals, in their own words

#	Competency
4	Wellness and crisis planning with the person
5	Navigating benefits, housing, employment, education and health systems
6	Teaching self-advocacy rather than doing for
7	Building natural supports and stepping back
8	Crisis recognition, de-escalation and warm hand-off
9	Documentation that is timely, individual and linked to a goal
10	Boundaries and dual-relationship disclosure
11	Trauma-informed practice
12	Cultural responsiveness and recovery-pathway neutrality
13	Staying within scope and referring
14	Medication limits (section 7)
15	Community and lone-worker safety
16	Mandated reporting

12. Qualifications

Lived experience of a mental health and/or substance use condition, and of recovery	Required. This is a bona fide occupational qualification for this position.
State peer credential	`[FILL IN exact credential name and issuing body]` — held at hire, or attained within `[FILL IN]` days
High school diploma or equivalent	Required, or `[FILL IN]`
Criminal background check, registry checks, exclusion screening	Required, completed before any independent contact with a person served
Valid driver's licence, acceptable driving record, insured vehicle	`[FILL IN Yes/No]` — required if transporting persons served
Basic computer skills to document in the record system	Required

Note on your recovery status: it is a qualification for the job. It is not recorded in clinical detail in your personnel file, it is not disclosed to persons served without your consent, and it is never used against you.

13. Supervision you will receive

- **Twice monthly** for your first 90 days, then **at least monthly**.
- Every session documented and signed by both of you.
- Covers your caseload, self-disclosure, boundaries, safety, documentation, your development — and how you are doing.
- You may request additional supervision at any time.

14. Acknowledgment

I have received and read this job description. I have had the chance to ask questions about it.

I can perform its essential functions with or without reasonable accommodation.

	Name	Signature	Date
Employee	_____	_____	_____
Supervisor	_____	_____	_____

Both signatures are required. A job description signed by the employee alone does not show that anyone explained it.

15. Annual review record

Review due	Completed (date)	Reviewed by	Changes made, or "reviewed, no change"	New version
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What a surveyor will ask a peer specialist

They will ask **you**, not the CEO — and if your answers and hers disagree, they believe yours.

They ask	What has to be true
"What is peer support? What is it not?"	You can say both halves in your own words
"Give me an example of sharing your story. Why did you share it?"	A real example with a purpose, where you were not the centre of it
"What do you do if someone says they stopped their medication?"	Section 7, in your own words
"You're at a home visit and feel unsafe. What do you do?"	"I leave." Nothing before that
"You see someone you serve at your own meeting. What now?"	Disclose it; you are a member in that room, not staff
"How do you know what a person's goal is?"	It is on their plan, in their words, and your notes link to it
"How do you take care of yourself? Does the agency help?"	A specific example, not "they say self-care matters"
"If you had a concern about the agency, who would you tell?"	A route you can name, including one past your own supervisor

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